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June 6, 2025

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Centers for Medicare & Medicaid Services
Department of Health and Human Services
200 Independence Ave, SW
Washington, DC 20201

Steven Posnack
Acting Assistant Secretary for Technology Policy (ASTP)
Office of the ASTP

Department of Health and Human Services
200 Independence Ave, SW
Washington, DC 20201

Re: CMS and ASTP Request for Information (RFI) on the Health Technology Ecosystem (CMS-0042-NC)

Dear Administrator Oz and Acting Assistant Secretary Posnack:

Established in 1943, the American Academy of Allergy, Asthma & Immunology (AAAAI) is a professional organization with more than 6,700 members in the United States, Canada and 72 other countries. This membership includes asthma, allergy and immunology (A/I), other medical specialists, allied health and related healthcare professionals—all with a special interest in the research and treatment of patients with asthma and allergic/immunologic diseases. We appreciate the opportunity to provide comments on the health technology ecosystem RFI and urge you to take action to address a longstanding public health issue: the inaccurate and persistent labeling of penicillin allergy in the electronic health record (EHR).

As outlined in AAAAI's 2023 Position Statement on Penicillin Allergy, mislabeling of penicillin allergy is both widespread and clinically consequential. Although approximately 10 percent of the U.S. population reports a penicillin allergy, more than 90 percent of these individuals are not truly allergic. Inaccurate allergy labels are often entered into the EHR without clinical confirmation and can remain indefinitely, even after subsequent tolerance is demonstrated or allergy evaluations are completed. This practice leads to routine substitution of broader-spectrum, less effective antibiotics, which increases the risk of antimicrobial resistance, *Clostridioides difficile* infection, treatment failures, and adverse events. It also drives up health care costs and contributes to longer and/or avoidable hospitalizations and complications.

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AAAAI's 2024 Position Statement on Changing Electronic Health Record Allergy Documentation to "Alerts" to Lead to Easily Understood, Actionable Labels underscores the critical need to shift EHR allergy documentation away from static labels and toward an "alert-based" model that is dynamic, understandable, and clinically actionable. Current EHR allergy fields often function as static lists that lack context, verification status, or test results—limiting their utility in guiding appropriate prescribing and clinical decision-making. Additionally, this information must be interoperable and available across health information exchanges (HIEs), so that accurate allergy histories can follow patients across care settings and avoid redundant testing or inappropriate antibiotic substitutions.

While not specific to questions posed in this RFI, we also encourage CMS to consider how preventive visits such as the Medicare Annual Wellness Visit (AWV) and the Initial Preventive Physical Examination (IPPE) could be leveraged to routinely prompt allergy review and facilitate delabeling in appropriate patients. These changes would not only improve the accuracy of clinical data, but also promote safer prescribing and reduce the use of inappropriate antibiotics. In 2024, Congress introduced the *Penicillin Allergy Verification and Evaluation (PAVE) Act* (H.R. 9778) to address this exact challenge. The legislation proposes to make penicillin allergy assessment a covered preventive service and to formally integrate allergy evaluation into Medicare preventive visits. AAAAI strongly supports the objectives of the PAVE Act and encourages CMS to pursue this reform using its existing authority.

CMS and ASTP have the opportunity to drive meaningful progress by modernizing EHR certification criteria and advancing health IT standards that support allergy-specific data fields, status indicators (e.g., ruled out, confirmed, uncertain), and seamless integration with clinical decision support tools. These changes would allow clinicians to more effectively engage in allergy reassessment and removing of drug allergy labels, ensuring patients receive evidence-based care informed by accurate, portable allergy information. Importantly, aligning allergy documentation with clinical realities would significantly strengthen antimicrobial stewardship and reduce the spread of antimicrobial resistance. ***We urge CMS and ASTP to take the necessary steps to prioritize and operationalize penicillin allergy delabeling as a core use case within the health technology ecosystem.***

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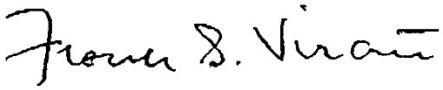
Thank you for the opportunity to provide comments on the aforementioned issues of importance to our members. Should you have any questions, please contact Sheila Heitzig, Director of Practice and Policy, at sheitzig@aaaai.org or (414) 272-6071.

Sincerely,



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